# IT DUE DILIGENCE REPORT

## TAMPA BAY SURGERY CENTER - MIDTOWN

Tampa, Florida

Prepared for Compass Surgical Partners
Prepared by: Benjamin Posner, vCIO - Dedicated IT
Assessment Date: 2026-01-12

# Executive Summary

**RECOMMENDATION: PROCEED WITH CONDITIONS**

Tampa Bay Surgery Center Midtown presents a significantly higher IT risk profile than its sister facility CASS. The center operates in an older multi-story building with legacy network infrastructure including Nortel switches (end-of-life for over a decade), Ubiquiti wireless (non-standard for Compass), an on-premise Microsoft Exchange server requiring migration, and disorganized wall-mounted IT equipment with poor cable management. The IT provider (Lance, same as CASS) was unavailable during the visit, leaving security controls entirely unverified. While the surgical operation itself is high-performing with strong pediatric volume and commercial payer mix, the IT infrastructure requires substantial investment to bring to Compass standards. This center is the primary focus of the deal and warrants a comprehensive stabilization plan.

| Critical Findings | High Findings | Est. CapEx | Est. Annual OpEx |
|---|---|---|---|
| 1 | 4 | $98,000 - $127,000 | $72,000 - $96,000 |

**Conditions for Proceeding:**

1. Mandatory follow-up meeting with Lance to inventory all systems, verify security controls, and document the Exchange server environment before close.
2. Seller to provide comprehensive IT credentials, vendor contacts, and documentation for all systems including the Exchange server, Nortel switches, and Ubiquiti wireless.
3. Budget allocation of $100,000-$130,000 CapEx for IT remediation must be factored into deal economics.
4. Day 1 and Week 1 security remediation plan must be agreed upon before close.

**Critical Pre-Close Requirements:**

- Obtain all admin credentials for Exchange server, Ubiquiti controller, Nortel switches, ISP equipment, and all applications.
- Obtain vendor contacts for Frontier ISP, Verizon, Avantix/SIS, VoIP provider, and any other IT vendors.
- Complete IT discovery call with Lance to fill documentation gaps.
- Verify Exchange server version, patch level, and mailbox count for migration planning.

# Facility Overview

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|---|---|
| **Facility Name** | Tampa Bay Surgery Center - Midtown |
| **Location** | Tampa, Florida (8-story building, ground floor surgery center, 8th floor offices) |
| **Approximate Staff** | ~30-40 (estimated from observation, high-volume center) |
| **Endpoints** | ~30-45 (estimated - workstations, tablets for anesthesia, printers, OR equipment) |
| **Operating Rooms** | 3 (ORs 1, 2, 3 referenced) |
| **Specialties** | Pediatric ENT (57% volume), Oculoplastics, Neurology, Orthopedics, Pain Management |
| **IT Contact** | Lance (same as CASS) - External IT; unavailable during visit. Manages multiple companies; This center is not his exclusive focus. |

# Findings Summary

| ID | Finding | Risk | Phase | Est. Cost |
|---|---|---|---|---|
| F001 | No Identified Firewall / UTM Appliance | **CRITICAL** | DAY1 | $6,700 |
| F002 | Legacy Nortel Network Switches (EOL) | **HIGH** | WEEK1 | $12,000 - $18,000 |
| F003 | On-Premise Exchange Server | **HIGH** | STABILIZE | $8,000 - $12,000 |
| F004 | IT Provider Dependency / No Documentation | **HIGH** | DAY1 | $10,000 |
| F005 | Security Controls Unverified (MFA/EDR/Backup) | **HIGH** | WEEK1 | $14,000 |
| F006 | Wireless Infrastructure Non-Standard / Mispositioned | **MEDIUM** | STABILIZE | $12,000 - $18,000 |
| F007 | Disorganized IT Infrastructure / No Rack | **MEDIUM** | STABILIZE | $5,000 - $8,000 |
| F008 | Multiple ISP Circuits / Unknown Configuration | **MEDIUM** | STABILIZE | $2,000 |
| F009 | Paper-Based Clinical Documentation | **LOW** | OPTIMIZE | N/A |

# Detailed Findings

## CRITICAL Risk Findings (1)

| F001: No Identified Firewall / UTM Appliance | |
|---|---|
| **Description** | Unlike CASS which has a Meraki MX84, no firewall or UTM appliance was identified during the Midtown site visit. The network closet contained Frontier ISP routers, Nortel switches, Ubiquiti wireless equipment, and Verizon fiber ONT, but no dedicated security appliance was observed. If no firewall exists, the facility's network is directly exposed to the internet with no intrusion prevention, content filtering, or network-level threat protection.<br>Note: A firewall may exist but was not visible during the visit. This must be verified with Lance. |
| **Business Impact** | Operational: Without a firewall, any internet-connected device is directly accessible from the public internet. This includes the Exchange server, any on-premise applications, and all endpoints. A single vulnerability could provide an attacker direct access to the network. Financial: Healthcare organizations without perimeter security face average breach costs exceeding $10M. Ransomware attacks on unprotected healthcare facilities typically result in 2-4 weeks of operational downtime. Compliance: A firewall is a fundamental HIPAA Security Rule requirement under the Access Control standard. |
| **Remediation** | 1. Verify firewall status with Lance immediately.<br>2. If no firewall exists: Deploy Meraki MX68 on DAY1 as emergency measure.<br>3. Configure firewall rules, VPN, content filtering, and intrusion prevention.<br>4. Implement network segmentation to isolate medical devices from general network. |
| **Timeline / Cost** | DAY1 - $6,700 (Meraki MX68 hardware $2,400 + 3yr license $1,500 + emergency implementation 16hrs @ $175/hr = $2,800) |
| **Evidence** | UNVERIFIED - No firewall observed; requires confirmation with Lance |

## HIGH Risk Findings (4)

| F002: Legacy Nortel Network Switches (End-of-Life) | |
|---|---|
| **Description** | A Nortel switch was identified in the IT closet with an IP address label (200.199.197.x). Nortel Networks went bankrupt in 2009 and was acquired by various companies. These switches have been end-of-life for over 15 years with no security patches, firmware updates, or vendor support available. Additionally, older Cisco Catalyst switches (appearing to be 3500/3700 series) were observed in the upstairs server room with disorganized cabling. |
| **Business Impact** | Operational: Nortel switches have known vulnerabilities that cannot be patched. A switch failure would require replacement with equipment that may not be configuration-compatible, extending downtime. Financial: Emergency replacement of failed legacy switches during surgical operations could cost $5,000-$10,000 in rush hardware and after-hours labor. Compliance: Running unsupported network equipment fails HIPAA risk assessment requirements and could result in audit findings. |

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| **Remediation** | 1. Replace all Nortel switches with Cisco Meraki MS series switches to align with Compass standards.<br>2. Replace aging Cisco Catalyst switches in upstairs rack.<br>3. Re-cable and document all network connections during replacement.<br>4. Implement VLANs for network segmentation (medical devices, staff, guest). |
| **Timeline / Cost** | WEEK1 - $12,000-$18,000 (2-3 Meraki MS switches $6,000-$12,000 + implementation 24hrs @ $175/hr = $4,200 + cabling remediation $2,000) |
| **Evidence** | OBSERVED - Nortel switch photographed in IT closet; legacy Cisco switches observed in upstairs rack |

| **F003: On-Premise Microsoft Exchange Server** | |
|---|---|
| **Description** | An on-premise server was identified upstairs that hosts Microsoft Exchange for email services. The specific Exchange version could not be determined during the visit, but given the age of other infrastructure, it is likely Exchange 2016 or older. On-premise Exchange servers are a primary target for attackers, with multiple critical vulnerabilities disclosed in recent years (ProxyLogon, ProxyShell, ProxyNotShell). Microsoft is actively pushing organizations to migrate to Exchange Online. |
| **Business Impact** | Operational: Exchange servers require constant patching and monitoring. If unpatched, exploitation is trivial using widely available tools. A compromised Exchange server provides an attacker access to all email, contacts, and calendars, plus a foothold for lateral movement to other systems. Financial: The 2021 Exchange ProxyLogon attacks affected over 250,000 servers globally; remediation costs averaged $50,000-$250,000 per organization. Compliance: Email containing PHI on an unpatched server is a HIPAA violation. Maintaining on-prem Exchange also requires ongoing server maintenance costs that exceed M365 subscription fees. |
| **Remediation** | 1. Verify Exchange version and patch level with Lance.<br>2. Plan migration to Microsoft 365 Exchange Online during stabilization phase.<br>3. Migrate all mailboxes (~40-50 estimated) to M365 Business Premium.<br>4. Decommission on-premise Exchange server after migration and verification.<br>5. Implement email security (Defender for Office 365) during migration. |
| **Timeline / Cost** | STABILIZATION - $8,000-$12,000 (~40-50 mailboxes @ $100/mailbox + base migration project $4,000) |
| **Evidence** | OBSERVED - Server identified upstairs; REPRESENTED - Staff confirmed Exchange server location |

| **F004: IT Provider Dependency / No Documentation** | |
|---|---|
| **Description** | Same IT provider dependency as CASS. Lance manages both facilities and was unavailable during the Midtown visit. Given the higher complexity of Midtown's infrastructure (legacy Nortel equipment, on-prem Exchange, Ubiquiti wireless, multi-floor building), the documentation risk is even more significant. No network diagrams, credentials, or system documentation were available. Staff indicated that Lance manages everything and they do not |

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| | have access to IT systems. The Day 1 takeover at Midtown will be substantially more complex than CASS. |
| **Business Impact** | Operational: The combination of legacy equipment, on-premise Exchange, mixed vendors, and no documentation creates a situation where any IT incident could result in extended downtime. The multi-floor building adds physical complexity. Financial: Emergency discovery and credential recovery at this facility could cost $10,000-$20,000 due to the complexity. Compliance: HIPAA requires documented IT processes; the current state is non-compliant. |
| **Remediation** | 1. Complete follow-up meeting with Lance pre-close with emphasis on Midtown systems.<br>2. Execute comprehensive Day 1 Takeover including physical survey, credential rotation, and full documentation.<br>3. Allow additional hours for Midtown takeover due to complexity (48+ hours vs 32-40 for CASS). |
| **Timeline / Cost** | DAY1 - $10,000 (48+ hrs @ $175/hr + PM; higher complexity than CASS) |
| **Evidence** | OBSERVED / REPRESENTED - Lance unavailable; staff confirmed they have no IT system access |

| **F005: Security Controls Unverified (MFA / EDR / Backup)** | |
|---|---|
| **Description** | As with CASS, no security controls could be verified during the visit. However, the risk is elevated at Midtown due to the presence of the Exchange server, legacy network equipment, and the higher patient volume. With an estimated 30-45 endpoints, the attack surface is larger. Anesthesia staff were observed using tablets, which adds another endpoint category that may or may not have security controls. |
| **Business Impact** | Operational: The combination of unverified security with known-vulnerable infrastructure (Nortel switches, potentially unpatched Exchange) creates a compound risk. Each gap amplifies the others. Financial: Given the higher volume and pediatric focus, breach notification costs would be significant. Compliance: Pediatric patient data receives additional scrutiny under HIPAA. |
| **Remediation** | 1. Verify current state with Lance.<br>2. Deploy MFA across all accounts with priority on Exchange admin and domain admin.<br>3. Deploy EDR to all endpoints including anesthesia tablets.<br>4. Implement Datto BCDR for server backup with Exchange-aware agent.<br>5. Implement email security with M365 migration. |
| **Timeline / Cost** | WEEK1 - $14,000 (MFA deployment $5,000 + EDR deployment $9,000 for ~40 endpoints) |
| **Evidence** | UNVERIFIED - No security controls confirmed during visit |

## MEDIUM Risk Findings (3)

### F006: Wireless Infrastructure Non-Standard and Mispositioned

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| **Description** | The facility uses Ubiquiti UniFi wireless access points, which do not align with the Cisco Meraki standard used across Compass facilities and at CASS. During the assessment, the assessor noted that APs 'need to be cleaned up because they are in the wrong positions and their mounts in wrong places, especially in the OR.' One AP was observed in a supply closet providing WiFi to the surgical area. A wireless site survey was identified as required. The AP positioning raises concerns about coverage, interference, and inappropriate placement in sterile OR environments. |
| **Business Impact** | Operational: Mispositioned APs can cause dead zones, interference, and unreliable connections for clinical applications. An AP in a supply closet is not designed to provide reliable OR coverage. Integration: Ubiquiti equipment cannot be managed through the Meraki dashboard, creating a split management environment. Compliance: Unreliable wireless in clinical areas could affect patient care documentation timeliness. |
| **Remediation** | 1. Conduct wireless site survey to map current coverage and identify gaps.<br>2. Replace all Ubiquiti APs with Cisco Meraki MR series.<br>3. Install APs in proper ceiling-mount positions per site survey.<br>4. Remove inappropriate AP placements (supply closets, etc.).<br>5. Configure separate SSIDs for medical devices, staff, and guest access. |
| **Timeline / Cost** | STABILIZATION - $12,000-$18,000 (Site survey $2,000 + 8-10 Meraki MR APs @ $950/each = $7,600-$9,500 + installation $2,400-$3,500 + cabling $2,000-$3,000) |
| **Evidence** | OBSERVED - Ubiquiti APs photographed; supply closet AP placement noted; assessor noted remediation required |

| **F007: Disorganized IT Infrastructure / No Proper Rack** | |
|---|---|
| **Description** | The Midtown IT infrastructure is wall-mounted in what appears to be a closet or utility space, with no proper server rack. Network equipment, patch panels, ISP equipment, and switches are mounted on the wall with cables running in a disorganized manner. The upstairs server room also has messy cabling around the Cisco switches and Exchange server. This is in stark contrast to CASS, which has a clean, purpose-built server closet with proper rack-mounted equipment. |
| **Business Impact** | Operational: Disorganized infrastructure makes troubleshooting difficult, increases the risk of accidental disconnection, and makes it harder to identify equipment during emergencies. Thermal management concerns with wall-mounted equipment in a closet. Financial: Every IT service call will take longer due to poor organization, increasing ongoing support costs. Integration: The current state makes documenting the network significantly more time-consuming. |
| **Remediation** | 1. Install proper wall-mount or freestanding rack appropriate for the space.<br>2. Re-cable all network connections with proper cable management.<br>3. Label all cables, ports, and equipment.<br>4. Document network topology during remediation.<br>5. Verify environmental conditions (temperature, ventilation) in IT spaces. |
| **Timeline / Cost** | STABILIZATION - $5,000-$8,000 (Wall-mount rack $500-$1,500 + cable management $2,000-$3,000 + labor 16-24hrs @ $175/hr) |

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| **Evidence** | OBSERVED - Wall-mounted equipment photographed; messy cabling documented in multiple photos |

| **F008: Multiple ISP Circuits / Unknown Configuration** | |
|---|---|
| **Description** | Two Frontier ISP devices and a Verizon fiber ONT were observed in the IT closet. The purpose and configuration of each circuit is unknown. It is unclear whether both circuits are active, whether they provide redundancy, or whether one is legacy. The Frontier equipment appeared to be DSL or low-bandwidth devices, while the Verizon ONT suggests a fiber connection. Without Lance's input, the ISP configuration cannot be assessed. |
| **Business Impact** | Operational: Unknown ISP configuration means the failover strategy (if any) is unknown. If one circuit is the primary and fails, it is unclear whether services would automatically switch. Financial: The facility may be paying for circuits that are not needed or not properly configured for redundancy. |
| **Remediation** | 1. Identify purpose and speed of each ISP circuit with Lance.<br>2. Determine which circuit is primary and whether failover exists.<br>3. Consolidate or properly configure ISP circuits for redundancy.<br>4. Ensure firewall (once deployed) is configured for dual-WAN if both circuits are retained. |
| **Timeline / Cost** | STABILIZATION - $2,000 (ISP assessment, configuration, and documentation - 12hrs @ $175/hr) |
| **Evidence** | OBSERVED - Frontier routers and Verizon ONT photographed in IT closet |

## LOW Risk Findings (1)

| **F009: Paper-Based Clinical Documentation** | |
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| **Description** | Similar to CASS, clinical documentation is primarily paper-based with scanning. Anesthesia has adopted tablets, which is a positive sign. The facility uses Avantix (SIS product) for scheduling and practice management. Lance has experience with HST, which facilitates the planned transition. |
| **Business Impact** | Same as CASS - efficiency opportunity during EMR/PM transition rather than an immediate risk. The existing tablet use by anesthesia demonstrates staff willingness to adopt digital workflows. |
| **Remediation** | Evaluate tablet-based documentation during HST transition. Build on existing anesthesia tablet adoption. |
| **Timeline / Cost** | OPTIMIZATION - N/A |
| **Evidence** | REPRESENTED - Staff described workflow; anesthesia tablets observed |

# Infrastructure Summary

## Network Infrastructure

| Component | Details |
|---|---|
| **Firewall** | NONE IDENTIFIED - Critical gap; verify with Lance |
| **Switches** | Nortel (EOL, 15+ years) + Cisco Catalyst (older models, upstairs rack) |
| **Wireless** | Ubiquiti UniFi APs (NOT Meraki); mispositioned; site survey required |
| **Internet** | Frontier (x2 devices - DSL?) + Verizon fiber ONT; speeds unknown |
| **Servers** | On-premise Exchange Server (upstairs, 8th floor) |
| **Workstations** | Mix observed; age and OS versions unverified |
| **Phone System** | Cisco VoIP (8800 series observed) |
| **Cameras** | Wired bullet cameras (not consumer-grade Arlo like CASS) |
| **Printers** | Kyocera multifunction printers (x2 observed) |

## Applications

| System | Solution | Hosting |
|---|---|---|
| EMR | Unknown - verify with Lance | Unknown |
| Practice Management | Advantix (SIS product) | Unknown - likely on-prem or hosted |
| PACS | None identified | N/A |
| Email | Microsoft Exchange (on-premise) | ON-PREM (8th floor server) |
| Anesthesia | Tablet-based system (vendor unknown) | Unknown |

# Security Controls

| Control | Status | Notes |
|---|---|---|
| Firewall / UTM | **NOT IDENTIFIED** | No firewall observed; CRITICAL gap |
| Multi-Factor Auth | **UNKNOWN** | Could not verify - Lance unavailable |
| Endpoint Detection | **UNKNOWN** | Could not verify |
| Email Security | **UNKNOWN** | On-prem Exchange likely has minimal security |
| Backup Solution | **UNKNOWN** | Exchange server backup unverified |

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|---|---|---|
| Physical Security | **DEPLOYED** | Wired bullet cameras observed; building access controls |

# Cost Estimates

## Capital Expenditures (Year 1)

| Item | Amount | Notes |
|---|---|---|
| Day 1 Takeover | $10,000 | 48+ hrs @ $175/hr + PM; higher complexity, multi-floor |
| Security Baseline | $14,000 | MFA ($5,000) + EDR ($9,000) for ~40 endpoints |
| Firewall Deployment | $6,700 | Meraki MX68 hardware + 3yr license + emergency install |
| Network Switch Replacement | $15,000 | 2-3 Meraki MS switches replacing Nortel + Cisco legacy |
| Exchange Migration | $10,000 | ~40-50 mailboxes @ $100/mailbox + base project $4,000 |
| Wireless Remediation | $15,000 | Site survey + 8-10 Meraki MR APs + install + cabling |
| Backup Enhancement | $12,000 | Datto BCDR appliance + implementation |
| Infrastructure Cleanup | $6,500 | Rack install, cable management, labeling, documentation |
| ISP Assessment | $2,000 | Circuit identification, configuration, optimization |
| Documentation | $7,000 | 40 hrs @ $175/hr - comprehensive documentation |
| **Subtotal** | **$99,200** | |
| Contingency (15%) | $14,880 | Higher contingency due to many unknowns |
| **TOTAL CAPEX** | **$114,080** | |

## Operating Expenditures (Monthly)

| Item | Monthly | Notes |
|---|---|---|
| Managed Services | $6,000 | ~40 users @ $150/user (Standard tier) |
| M365 Licensing | $1,000 | ~40 users @ $25/user (Business Premium) |
| Backup Service | $1,000 | Datto BCDR monthly service (higher data volume) |
| Security Tools | $520 | MFA ($4/user) + EDR ($7/endpoint) for ~40 users/endpoints |
| Meraki Licensing | $150 | Switches + APs annual licensing / 12 |
| **TOTAL MONTHLY** | **$8,670** | |

| | | |
|---|---|---|
| **TOTAL ANNUAL** | **$104,040** | |

## Follow-Up Items

| Item | Owner | Due Date | Status |
|---|---|---|---|
| Schedule follow-up call with Lance (PRIORITY) | Jay / Ben Posner | Pre-Close | **PENDING** |
| Verify firewall existence and configuration | Lance / Ben | Pre-Close | **PENDING** |
| Determine Exchange version and mailbox count | Lance / Ben | Pre-Close | **PENDING** |
| Obtain all IT credentials and vendor contacts | Jay / Lance | Pre-Close | **PENDING** |
| Identify ISP circuits, speeds, and contracts | Lance / Jay | Pre-Close | **PENDING** |
| Verify MFA / EDR / Backup status | Ben Posner | Pre-Close | **PENDING** |
| Verify workstation OS versions (Win 7 concern) | Lance / Ben | Pre-Close | **PENDING** |
| Wireless site survey | Dedicated IT | Week 1 | **PENDING** |

## Deal Risks

- IT remediation costs ($100K-$130K CapEx) are significantly higher than a typical ASC acquisition due to legacy infrastructure. This must be factored into deal economics.
- If Lance remains uncooperative, credential recovery and system discovery at Midtown will be substantially more complex than at CASS due to legacy equipment and on-prem Exchange.
- The potential absence of a firewall represents an active security risk that exists today. If a breach occurs between diligence and close, liability implications could be significant.
- Workstation OS versions were not verified. If Windows 7 or other EOL operating systems are in use (noted as a concern in previous conversation), hardware replacement costs of $15,000-$25,000 could be required.
- The multi-floor building (ground floor surgery, 8th floor offices/server) creates complexity for network management, cabling, and physical access to IT equipment.
- On-premise Exchange is an active security liability. Every day it remains unpatched and exposed increases the probability of compromise.

## Positive Observations

✓ High-volume surgical center with 57% pediatric mix and strong commercial payer mix. The business case is strong.

- ✓ Cisco VoIP phones (8800 series) are modern and enterprise-grade, suggesting some recent IT investment.
- ✓ Anesthesia has already adopted tablet-based documentation, demonstrating staff willingness to embrace digital workflows.
- ✓ Lance has experience with HST, which facilitates the planned PM/EMR transition.
- ✓ Wired security cameras are more reliable than the consumer-grade Arlo cameras at CASS.
- ✓ Efficient patient flow design noted by clinical staff, supporting high-volume operations.
- ✓ Kyocera MFPs are enterprise-grade printing equipment.

**Prepared by:**

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Benjamin Posner, vCIO - Dedicated IT
Date: 01/12/2026